Harbor Point Mutual Auto Insurance

Bodily Injury Unit

Claim Notes

Claim number:	C-1420
Claimant:	Pamela Church
Adjuster:	Alison Sweeney

02/17/2025
Claimant reported a left turn collision at an intersection and states the other vehicle struck the driver's side as the claimant was completnig the turn. Claimant is not represented by counsel and has given a recorded statement.

02/20/2025
Police report was obtained and lists the claimant as the driver of the insured vehicle, with the other driver cited for failing to yield. The driver is named on the policy and coverage is confirmed.

03/08/2025
Two independent witnesses at the scene were interviewed by phone and both describe the impact in the same sequence as the claimant. Scene photos were received and show damage consistent with a side impact.

04/04/2025
Adjuster inspected the insured vehicle at the repair facility. The damage pattern lines up with the described impact, and no prior unrelated damage was noted during the inspection.

04/13/2025
Claimant reports pain in the right shoulder and limited range of motion following the collision and was referred for an initial medical evaluation. Medical records indicate the injury type and force of the crash are a good match.

05/16/2025
Claimant's medical history shows treatment for the same shoulder area within the last two years, and the treating provider notes that this earlier conditoin contributes to some of the current complaints. Adjuster has noted this for the injury evaluation.

06/21/2025
A neighbor contacted the office and said they believed the vehicle had been damaged a few weeks before the reported loss. No photos or other support were provided, and the claim file has been flagged for follow-up with the repair records.

06/29/2025
Treatment has followed a typical course of care, with frequency of visits tapering as the claimant improved. The claimant has given the same account of events in every conversation with the adjuster and the medical team.

06/30/2025
Claimant is improving and is expected to return to normal activity shortly. Injury claim is under review pending the prior-condition apportionment and the outcome of the follow-up on the neighbor's report.

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Recorded Statement

Claim number:	C-1420
Statement of:	Pamela Church
Taken by:	Alison Sweeney
Date:	01/24/2025

Q: Thank you for taking the time to speak with me today. This recorded statement concerns the collision you reported. Please state your full name for the record.

A: I'd rather just keep this to the accident itself, if that's okay.

Q: That's fine, though I do need your name for the file. You can give just the first name if you prefer.

A: Okay, it's the claimant, you have it on the form already.

Q: Understood. Can you walk me through where you were heading when the collision happened?

A: I was in the left-turn lane at a signalized intersection in Houston, giong to turn left onto the cross street. The light was green for my direction, but the left arrow had not come on yet.

Q: So you were waiting for the arrow to turn left. What did you see next?

A: I started to creep forward into the intersection to get a better view of oncoming traffic. Then I saw a car coming from the opposite direction and I thought I had time. I started my turn and that's when it hit my driver's side front.

Q: Did the other driver have a chance to stop before impact?

A: I honestly don't know. It happened fast. I heard the brakes after I had already committed to turning.

Q: Where do you feel injured?

A: My right shoulder. I hit the door when we spun a little and it's been sore and hard to lift my arm since. I went to urgent care that evening and they took some X-rays.

Q: Have you seen a doctor since then or started any treatment?

A: I have a follow-up scheduled and I'm doing physical therapy exercises at home for now.

Q: Thank you. Is there anything you would like to add to your account of the collision?

A: Just that I was trying to be careful, but I know I should have waited for a clearer gap before turning.

Q: Thank you. This concludes the recorded statement.